

GLIX[®] TABLET 5mg

Glipizide, the active content of Glix Tablet, is the most rapid and short-acting blood glucose lowering drug of the sulfonylurea class. Due to its rapid onset of action, glipizide is most able to bring about acute insulin release and, because of its short duration of action, it is not liable to provoke long-lasting hypoglycaemia.

Ingredient(s):

Each tablet contains :

Glipizide 5mg

Pharmacodynamic:

1. Glipizide is an oral blood-glucose lowering drug belonging to the sulfonylurea class. It appears to lower the blood glucose acutely by stimulating the release of insulin from pancreas.
2. Glipizide may be an effective alternative in some patients who have not responded or have ceased to respond to other sulfonylureas.
3. In patients with NIDDM, glipizide therapy was effective in controlling blood sugar without deleterious changes in their plasma lipoprotein profiles.

Pharmacokinetic:

1. Gastrointestinal absorption of glipizide is rapid and essentially complete. Peak plasma concentrations occur in 1 ~ 3 hours after a single oral dose. The elimination half life ranges from 2 ~ 4 hours in normal subjects. Although absorption was delayed, the total absorption and disposition of an oral dose was unaffected by food in normal volunteers.
2. Glipizide is extensively metabolized which occur mainly in the liver. The primary metabolites are inactive and are excreted mainly in the urine.

Indication(s):

For control of hyperglycaemia and its associated symptomatology in patients with non-insulin dependent diabetes mellitus after an adequate trial of dietary therapy has proved unsatisfactory.

Dosage and Administration:

The minimum effective dose for the patient is determined based on the usual monitoring of urinary glucose and periodical monitoring for the patient's blood glucose. Initially, the recommended dose is 2.5~5 mg daily given approximately 30 minutes before meal. Dosage should be adjusted according to response; up to 15mg may be given as a single dose before breakfast. The maximum daily dosage for Glix tablet is 40mg. Doses over 15mg should usually be divided.

When transferring patients from insulin to Glix Tablet, subsequent reductions in insulin should depend on individual patient response.

To be dispensed on physician's prescription.

Route of administration:

Oral

Contraindications:

1. Glix Tablet is contraindicated in patients with known hypersensitivity to glipizide.
2. It is contraindicated in diabetic ketoacidoses with or without coma. In such a case, insulin should be the appropriate treatment.
3. Juvenile, growth onset or brittle diabetes mellitus.
4. Severe renal and hepatic insufficiency.

Precaution(s) / Warning(s):

1. Patients with impaired renal and/ or hepatic function may have slower metabolism and excretion of glipizide.
2. Hypoglycaemia may occur when caloric intake is deficient, after severe or prolonged exercise, when alcohol is ingested, or when more than one glucose-lowering drug is used.
3. Pregnancy: There are no adequate and well controlled studies in pregnant women. Glipizide should be used during pregnancy only if potential benefit justifies the potential risk to the fetus.
4. Nursing mother although it is not known whether glipizide is excreted in human milk, some sulfonylurea drugs are known to be excreted in human milk. Therefore, a decision should be made whether to discontinue nursing or to discontinue the drug, taking into account the importance of the drug to the mother.

Drug Interactions:

The Hypoglycaemic action of sulfonylureas may be potentiated by certain drugs including non-steroidal anti-inflammatory agents and other drugs that are highly protein bound, salicylates, sulfonamides, chloramphenicol, probenecid, monoamine oxidase inhibitors, and beta-adrenergic blocking agent. When such drugs are administered to a patient receiving Glix, the patient should be observed closely for hypoglycaemia. When such drugs are withdrawn from a patient receiving Glix, the patients should be observed closely for loss of control.

In vitro binding studies with human serum protein indicates that Glix binds differently than tolbutamide and does not interact with salicylates or dicumarol. However, cautions must be exercised in extrapolating these binding to the clinical situations and in the use of Glix with these drugs.

Side Effect(s) / Adverse Reaction(s):

1. Hypoglycaemia: Episodes may occur during treatment with sulfonylureas, especially in debilitated or elderly subjects, after unusual physical exertion, when food intake is irregular or alcoholic beverages are taken, when kidney and/ or liver function is impaired.
2. Gastrointestinal: Gastrointestinal side effects such as nausea, diarrhea, constipation and gastralgia are the most common and appear to be dose-related. They may disappear on division or reduction of dosage.
3. Dermatological: Allergic skin reactions, such as erythema morbilliform or maculopapular eruptions, urticaria, pruritus and eczema, may occur. These may be transient and may disappear despite continued use of glipizide.
4. Hematological: leukopenia, agranulocytosis, thrombocytopenia, hemolytic anemia, aplastic anemia, and pancytopenia have been reported with the use of sulfonylureas.
5. Others: Dizziness, drowsiness and headache have been rarely reported. They are usually transient and seldom.

Symptoms and Treatment for Overdosage, and Antidote(s):

There is no well-documented experience with glipizide overdose. Generally, overdose with sulfonylureas can produce hypoglycaemia symptoms without loss of consciousness or neurologic findings should be treated aggressively with oral glucose and adjustments in drug dosage and/ or meal patterns. Severe hypoglycaemic reactions with coma, seizure or other neurological impairment occur infrequently. If hypoglycaemic coma is diagnosed or suspected, the patient should be given a rapid intravenous injection of concentrated (50%) glucose solution. This should be followed by a continuous infusion of a more dilute (10%) glucose solution at a rate that will maintain the blood glucose at a level above 100mg/dL.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):

A white to off-white color round tablet, one side with a score, and other side with a mark " G ", "   ".

Blister packing of 10's x 10 and 10's x 50.



Manufacturer and Product Registration Holder:
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